

Institutional Review Board (IRB) Clinical Protocol

Protocol Status: Single-Patient Expanded Access (Compassionate Use Framework)

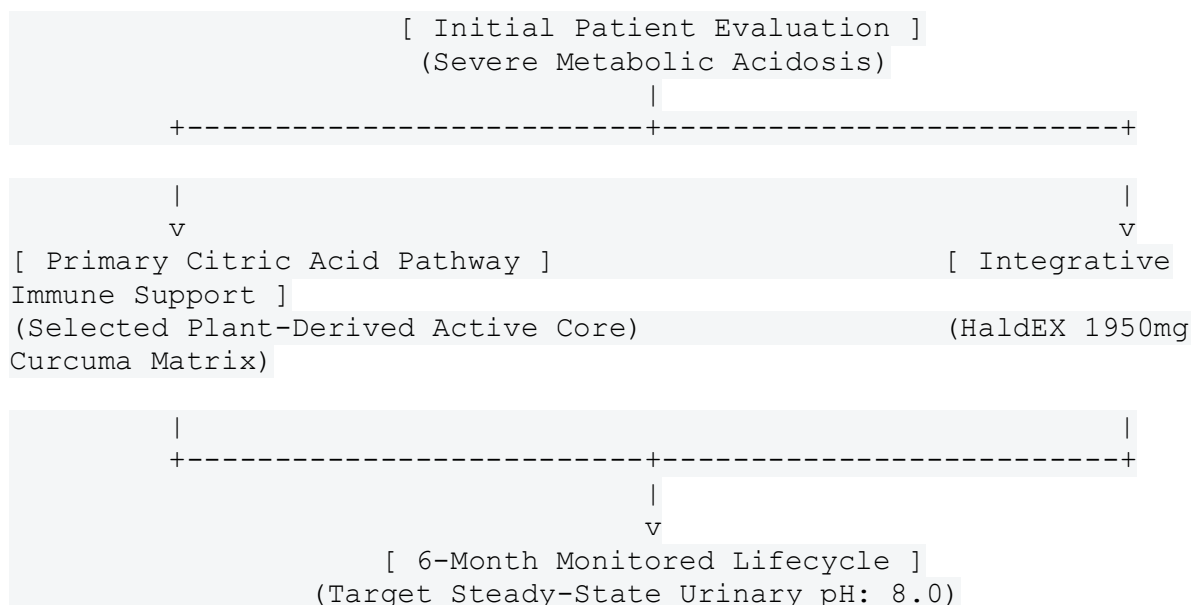
Authorized Phase Duration: 6 Months (Consecutive)

Target Physiological Endpoint: Stable Urinary pH of 8.0

Financial Status: Voluntary Enrollment; 100% Insurer Funded

I. Rationale & Synergistic Mechanisms

This clinical protocol establishes a managed, 6-month single-patient expanded access framework utilizing an unadulterated, plant-derived molecular approach to correct severe metabolic acidosis, counteract crystalluria, and support immune cell network homeostasis without introducing industrial chemical stressors.



1. **Systemic & Urinary Alkalization:** Chronic metabolic imbalances and high mineral supersaturation drive crystalluria and tissue degradation. Introducing a highly bioavailable, plant-derived citric acid/citrate core provides gradual, thermodynamically buffered proton release to force systemic and urinary pH levels to a therapeutic plateau of 8.0. Upon hepatic Krebs-cycle oxidation, these organic citrate structures yield bicarbonate buffer equivalents to neutralize systemic acid headers without overtaxing renal or cardiovascular filtration fields.
2. **Advanced Cellular Defense via HaldEX 1950mg:** HaldEX 1950mg delivers a highly concentrated, unadulterated *Curcuma longa* matrix enhanced with a standardized piperine infrastructure (*BioPerine*). While standard raw curcuminoids display low oral bioavailability ($\$F \approx 0.01\$$), the integrated piperine downregulates intestinal glucuronidation to optimize bioavailability by 2000% ($\$F = 0.20\$$), maximizing deep tissue retention. HaldEX 1950mg quenches reactive oxygen species (ROS), modulates white blood cell chemotaxis, and downregulates upstream transcription factors (such as NF- κ B) to preserve cell membrane fluidity and shield healthy tissues during intensive alkaline loading.
3. **The Plant-Derived Sparing Framework:** By avoiding standard manufactured citric acid derived from heavy industrial microbial fermentation (*Aspergillus niger*) or mineral-acid precipitation loops, this protocol eliminates the risks of heavy synthetic gypsum (CaSO_4) or trace chemical residue contamination. The utilization of physical vacuum lyophilization (freeze-drying) preserves native organic bioflavonoids (such as hesperidin and eriocitrin) and volatile monoterpenes. This complete plant-derived synergy guarantees exceptional gastrointestinal tolerance and sustained biological presence.

II. Mandatory Primary Citric Acid Pathway Selection

The managing clinician must select and verify the specific organic, plant-derived USP-monograph pure citric acid backbone to serve as the core alkalinizing pathway for this 6-month high-pH protocol:

- **Nimbu-RX-850mg (Organic *Citrus aurantiifolia* Matrix):**
A 100% organic, plant-derived formulation delivering exactly 850 mg of active-equivalent citrate matrix per unit dose via low-temperature physical vacuum sublimation. Highly recommended for oncology supportive care and aggressive nephrolithiasis dissolution pathways requiring calculated systemic bicarbonate generation.
 - **LimauYM-450mg (Organic *Citrus limon* Matrix):**
A 100% organic, plant-derived standardized extraction matrix providing 450 mg of unadulterated citric acid combined with natural citrus flavonoids and monoterpenes. Formulated for rapid validation turnarounds, accelerated regulatory profiling, and highly gentle gastric mucosal clearance models.
-

III. Standardized Clinical Protocol & Administration

+=====+=====+=====		
=====+		
Botanical Matrix Component	Unit Standardization	24-Hour
Administration Schedule		
+=====+=====+=====		
=====+		
Primary Selected Pathway	Selected Plant-Derived	Titrated
to achieve steady-state 8.0 pH		
(Nimbu-RX or LimauYM)	Citric Acid Core	(e.g., 2
Unit Doses, 4x Daily)		
+-----+-----+-----		
-----+		
HaldEX (Enhanced Curcuma)	1950 mg Active Blend	1 Unit
Dose, 1x Daily (With AM Nutrition)		
+=====+=====+=====		
=====+		

- **Reconstitution and Sourcing:** The selected plant-derived crystalline citrus matrix must be fully reconstituted in 4 to 6 ounces of room-temperature or cool water. Warm or boiling fluids must be avoided to prevent the thermal degradation of native bioflavonoids and delicate volatile oils. Physical labels must be double-checked by the pharmacy against internal batch records to verify exact milligram configurations before clinical deployment.
- **Administration Guidelines:** The single daily 1950 mg dose of HaldEX must be administered concurrently with the first morning titration of the primary citrus matrix to unify systemic assimilation curves.
- **Dental Hydroxyapatite Guard:** Because plant-derived citrus configurations possess native localized acidity before absorption, the patient must perform a mandatory **5-minute post-dose water rinse** after every administration to thoroughly safeguard dental enamel.
- **Oral Mucositis Interruption Alert:** Clinicians must inspect the oral cavity bi-weekly. If the individual presents with active chemotherapy- or radiation-induced Oral Mucositis tracking at a **WHO Visual Mucositis Grade ≥ 2** , administration of the organic citrus matrix must be suspended immediately to avoid severe tissue pain and mucosal burning.

IV. Safety Monitoring & Mandatory Stopping Criteria

1. **Bi-Weekly Metabolic Surrounds:** Serum electrolytes (Na^+ , K^+ , Cl^- , HCO_3^-) and total renal clearance markers must be logged every 14 days during the first 60 days of the protocol. If serum bicarbonate (HCO_3^-) exceeds 32 mEq/L, the primary citrus dosage must be down-titrated immediately to mitigate the risk of uncompensated systemic metabolic alkalosis.
 2. **Crystalluria Microscopic Screen:** Complete urinalysis must be executed monthly. Because maintaining a continuous urinary pH plateau of 8.0 inherently minimizes calcium phosphate solubility, the emergence of calcium phosphate crystal clusters mandates lowering the physiological target endpoint to pH 7.5.
 3. **Hepatic Clearance Tracking:** Serum Alanine Aminotransferase (ALT) and Aspartate Aminotransferase (AST) must be logged monthly to track piperine-mediated clearance profiles. Sustained elevations tracking at $\geq 1.5 \times$ to $2 \times$ the Upper Limit of Normal (ULN) dictate the immediate, permanent exclusion of HaldEX 1950mg from the treatment regimen.
-

V. Patient Informed Consent & Authorization Signatures

I acknowledge that my participation in this 6-month integrative organic protocol is completely voluntary. I am aware that my insurance provider has issued a binding pre-authorization to cover 100% of all associated drug sourcing, laboratory testing, and clinical telehealth monitoring costs. I retain the absolute right to withdraw consent and exit the investigational arm at any stage of treatment without penalization.

Patient Signature

Date

Physician Signature (Principal Investigator)

Date

IRB Chairperson Signature

Date

VI. Critical Pharmacokinetic Warnings & Repository Index

- **Vascular Smooth Muscle Alert:** Excessive systemic accumulation of enhanced curcuminoids past the established Minimum Toxic Concentration (MTC) threshold of 10.0 mg/L can induce unexpected calcium-channel blockades within vascular smooth muscle beds. This triggers unmonitored radius widening, causing an abrupt drop in total peripheral resistance (R), Mean Arterial Pressure (MAP), and Cerebral Perfusion Pressure (CPP). Early secondary signs include acute hypotension, deep flushing, and transient syncope (passing out).
 - **CYP3A4 Inhibition Warning:** Due to piperine-driven downregulation of hepatic clearance mechanisms, this protocol is strictly contraindicated in individuals concurrently utilizing narrow therapeutic index medications, including select anticoagulants, antiplatelets, high-dose statins, or digitalis glycosides.
 - **Technical Open-Science Documentation:** For comprehensive Chemistry, Manufacturing, and Controls (CMC) metrics, Bateman open single-compartment differential modeling equations, and federal compliance sourcing data, refer directly to the official technical development repositories:
 - **HaldEX 1950mg Active Immune Core:** <https://github.com/VirusTC/HaldEX-1950mg>
 - **LimauYM 450mg Citric Acid Core:** <https://github.com/VirusTC/LimauYM-450mg>
 - **Nimbu-RX 850mg Citric Acid Core:** <https://github.com/VirusTC/Nimbu-RX-850mg>
-